WayMed · Guidance Network

How a first-time patient gets help at Civil Hospital Karachi, from the gate to the right department, using our web app.

Our team
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How we got here design thinking, step by step

1. **We visited Civil Hospital Karachi** and talked to 6 people: 3 patient attendants (from Thatta, Sanghar and a father of a 7-year-old), a child specialist, a doctor's assistant and a nurse intern.
2. **34 sticky notes, 5 problem themes**, each a possible "How might we": no safe place to rest · lost without guidance · left in the dark · money and lost work · an overstretched system.
3. **We finalized one HMW** (theme "lost without guidance"):

HMW: How might we help first-time visitors reach the right department the first time?

4. **We compared 4 idea groups** for advantages, disadvantages, usability and scale, then finalized an "Intelligent Hospital Navigation System". We kept what needs no new hardware and dropped what hurts privacy or costs too much:

Sign boards & displays · Phone, QR & web guidance · Smart slip (QR token) · Kiosks & boards · Voice assistant · ~~Camera tracking~~ · ~~Guide robots~~

Our idea, raw as we first wrote it, by hand

ONE SENTENCE

A web app on the patient's own phone: scan the QR at the gate, pick Urdu, Sindhi or English, get a slip with a token, and it shows the way to the right department step by step and tells you when it is your turn. No camera, no account.

↓ This idea became the WayMed web app

Works on any phone browser, at a kiosk, and on the entrance board. Every screen in **Urdu, Sindhi and English** with icons, colours and a Listen button. A red **Emergency** button is on every screen.

Live demo: **waymed-nvfl.vercel.app** (sample hospital data, clearly marked).

Step	What the patient does in the app	What happens in the hospital	Problem solved (from our interviews)
1 · Before coming at home	Opens a shared link: what to bring, opening hours, "only one or two attendants". Can book a time.	Nothing yet: the family arrives prepared.	**S1:** *"patients arrive with little early guidance"* · **D1:** 6 to 9 attendants crowd each child
2 · At the gate Main Entrance	Scans the QR poster, picks a language, taps "I am at the hospital gate". Sees the 5 parts of the visit.	A QR poster at the gate. The guard only points to it.	**P1:** *"did not know where to go or whom to ask"*
3 · Slip Room Registration	Chooses the need (Check-up, Child, Women, Eye…) and gets a slip with a token. No name, no phone number.	The desk issues or prints the slip. The department already sees the token coming.	The right department is chosen **before** walking, not by trial and error.
4 · Walk to the department	Follows one "next step" card and a route in the department's colour and shape. Location optional, or taps "I reached …".	Colour-coded signs and a QR sticker at every landmark.	**P2:** one person said *"emergency no. 3"*, another sent him elsewhere
5 · Wrong turn anywhere	The phone warns "turn back", or the next QR sticker shows "You are here" and a new route. "I feel lost" calls a guide.	Staff receive the guide request with the place.	**P2:** *"hours lost while the child got worse"*
6 · Wait near the room	Sees people ahead and a time range, and the nearest water, washroom and seats. The family can follow the same token.	The entrance board shows "now serving". Staff press Next.	**P2:** *"no news from inside"* · **P1:** no place to sit
7 · Your turn doctor's room	The phone turns green and vibrates: "Your turn now, go in".	The doctor's room calls the token.	No crowding at the door to ask "is it my turn?"
8 · After check-up going home	Gets a follow-up card in their language: next date, bring reports, come to Emergency if worse.	Doctor or desk may write the date; printing is optional.	**P1:** unclear follow-up after discharge · **S1:** follow-up gap
Emergency any moment	Presses the red Emergency button: one fastest route from where they stand, no slip needed.	Straight to Emergency; staff are told "Emergency".	**P2:** child unwell, sent between places

Who does what

- **Patient / family:** scan, choose, follow, wait, go in.
- **Hospital:** QR posters and stickers, slip desk, "Next" button, entrance board.

What it does not do

- No medical advice or diagnosis.
- No camera tracking, no accounts, no names stored.
- Cannot make the lab or doctors faster.

How we will know it works

- First-time-right rate (one-tap question).
- Time from gate to department.
- Pilot: one department, two weeks.

Evidence codes: P1 Khurshid (attendant, Thatta) · P2 Shams (father of a 7-year-old) · D1 Dr. Tahira (child specialist) · S1 Abdullah (doctor's assistant). Interview points are paraphrased, not measured.

Floor plan, doctors and timings in the demo are sample data.